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Correct Answer

Practice Exam - 1

Next Question

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A 36-year-old man is referred for evaluation and management of hypertrophic cardiomyopathy. Echocardiogram showed asymmetric septal hypertrophy (interventricular septum 2.2 cm, posterior wall 1.2 cm) with ejection fraction of 65-70%, systolic anterior motion of the anterior mitral leaflet, left ventricular outflow gradient of 55 mmHg at rest, 1+MR, and no apical aneurysm. Holter monitor shows no atrial fibrillation, 1% premature ventricular contraction burden, and occasional ventricular couplets. He takes metoprolol XL 100 mg daily. He walks 2 miles per day and swims occasionally with his children without exertional dyspnea or lightheadedness. He has not had syncope. A paternal uncle had diabetes and died of a heart attack at age 49. Otherwise, there is no known family history of cardiac disease.

Which of the following is the next best step in management

A. Refer for an implantable cardioverter defibrillator



B. Refer for genetic testing

C. Recommend septal myectomy

D. Recommend reduction in physical activity

Commentary

References

Testing Point: The Board-certified AHFTC specialist should know when it is appropriate to refer patients with hypertrophic cardiomyopathy for genetic testing.

Answer: B. Refer for genetic testing. Offering genetic testing to patients with hypertrophic cardiomyopathy (and other inherited heart disease) is recommended (class I) to elucidate the genetic basis, evaluate for phenocopies, and facilitate screening of at-risk family members (typically first degree relatives). This patient has multiple family members at risk who should be screened traditionally or via cascade testing if a pathologic variant is found.

Answer A: Primary prevention defibrillators are appropriate for some patients with hypertrophic cardiomyopathy, but this patient does not meet major (extreme hypertrophy, family history of SCD likely attributable to HCM, left ventricular dysfunction, or apical

aneurysm) or minor criteria (episodes of non-sustained ventricular tachycardia, unexplained syncope, or extensive scar burden) per 2020 AHA/ACC guidelines. The death of a close family member < 50 years warrants investigation of the cause, but there is a plausible alternative here.

Answer C: This patient has good functional status (NYHA 1), so septal reduction therapy is not recommended, even with a resting gradient >50 mmHg.

Answer D: High-intensity competitive athletics may warrant individualized discussion, but moderate recreational activity is encouraged.

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